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Recommendations for the Conduct, Reporting, Editing, and Publication of Scholarly Work in Medical Journals
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1. Covering letter
2. Manuscript including tables and panels
3. Figures
4. Author statement form (see next section)
5. Declaration of interests and source of funding statements (see next section)
6. In-press papers—one copy of each with acceptance letters
7. Protocols and CONSORT details for randomised controlled trials (see Articles)
8. We encourage disclosure of correspondence from other journals and reviewers, if previously submitted, and we might contact relevant editors of such journals
9. Research in context panel, for all primary research Articles

Covering letter

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ICMJE Recommendations
http://www.icmje.org

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paper, please provide a separate document with a table of first names and surnames of all members of the group (this is to ensure that PubMed and similar databases encode the names correctly).

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Reporting sex-based and gender-based analyses

Elsevier's author guide
https://www.elsevier.com/en-gb/researcher/author/policies-and-guidelines/edi

The *Lancet* Group's commitment to equity, diversity, and inclusion
https://www.thelancet.com/equity-diversity-inclusion/commitments

Reporting guidance

For research involving or pertaining to humans, animals, model organisms, or eukaryotic cells, investigators should integrate sex-based and gender-based analyses into their research design according to evolving funder/sponsor requirements and best practices within a field. Authors should address their research's sex and/or gender dimensions in their manuscript. In cases where they cannot, they should discuss this as a limitation to their research's generalisability. With research involving cells and model organisms, researchers should use the term "sex". With research involving humans, researchers should consider which terms best describe their data (see Definitions section below). Authors can refer to the Sex and Gender Equity in Research (SAGER) Guidelines and the SAGER guidelines checklist. They offer systematic approaches to the use and editorial review of sex and gender information in study design, data analysis, outcome reporting, and research interpretation. However, there is no single, universally agreed-upon set of guidelines for defining sex and gender or reporting sex-based and gender-based analyses.

Sex and Gender Equity Research (SAGER) Guidelines
https://researchintegrityjournal.biomedcentral.com/articles/10.1186/s41073-016-0007-6

SAGER guidelines checklist
https://ese.arphahub.com/article/86910/

Definitions

In human research, the term "sex" carries multiple definitions. It often refers to an umbrella term for a set of biological attributes associated with physical and physiological features (eg, chromosomal genotype, hormonal levels, internal and external anatomy). It can also signify a sex categorisation, most often designated at birth ("sex assigned at birth") based on a newborn's visible external anatomy. The term "gender" generally refers to socially constructed roles, behaviours, and identities of women, men, and gender-diverse people that occur in a historical and cultural context, and might vary across societies and over time. Gender influences how people view themselves and each other, how they behave and interact, and how power is distributed in society. Sex and gender are often incorrectly portrayed as binary (female/male or woman/man), concordant, and static. However, these constructs exist along a spectrum that includes additional sex categorisations and gender identities, such as people who are intersex/have differences of sex development (DSD), or identify as non-binary. In any given person, sex and gender might not align, and both can change. Sex and gender are not entirely discrete concepts and their definitions continue to evolve. Biology and society influence both, and many languages do not distinguish between them. Since the terms "sex" and "gender" can be ambiguous, authors should describe the methods they use to gather and report sex-related and/or gender-related data (eg, self-report or physician-report, specific biological attributes, current sex/gender, sex assigned at birth, etc) and discuss the potential limitations of those methods. This will enhance the research's precision, rigor, and reproducibility, and avoid ambiguity or conflation of terms and the constructs to which they refer. Authors should use the term "sex assigned at birth" rather than "biological sex", "birth sex" or "natal sex" as it is more accurate and inclusive. When ascertaining gender and sex, researchers should use a two-step process: (1) ask for gender identity allowing for multiple options and (2) if relevant to the research question, ask for sex assigned at birth. In addition to this defining guidance and the SAGER guidelines, you can find further information about reporting sex and gender in research studies in The *Lancet* Group's commitment to equity, diversity, and inclusion here

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In the Discussion section, please discuss the representativeness of the study population, to help readers assess the applicability of the findings to their setting.

Because race is a sociocultural construct, not a fixed biological trait, we ask authors to avoid use of race-based reference ranges and algorithms. For all manuscripts, any discussion of data in relation to race, ethnicity, or both should consider the wider context of socioeconomic, historical, and other structural drivers, for which race or ethnicity might be surrogate measures. We ask authors to qualify race-based associations drawn from observational data by discussing the potential limitations of such data and the possible role of unmeasured confounders. Such explanation can reduce the potential for harm from simplistic attributions to race.

We also ask authors to consider a strengths-based approach rather than a deficit discourse, when discussing findings related to race and ethnicity. For instance, discuss how findings might promote health and wellbeing, rather than focusing on problems.

Strengths-based approach
https://www.lowitja.org.au/wp-content/uploads/2023/05/deficit-discourse-strengths-based.pdf

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Red section (Articles)

Articles

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- Use the recommended international non-proprietary name (rINN) for drug names. Ensure that the dose, route, and frequency of administration of any drug you mention are correct
- Use gene names approved by the Human Gene Organisation. Novel gene sequences should be deposited in a public database (GenBank, EMBL, or DDBJ), and the accession number provided. Authors of microarray papers should include in their submission the information recommended by the MIAME guidelines. Authors should also submit their experimental details to one of the publicly available databases: ArrayExpress or GEO
- Include any necessary additional data as part of your EM submission
- All accepted Articles should include a link to the full study protocol published on the authors' institutional website (see *Lancet* 2009; **373:** 992 and *Lancet* 2010; **375:** 348)
- We encourage researchers to enrol women and ethnic groups into clinical trials of all phases, and to plan to analyse data by sex and by race

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- All research papers (including systematic reviews/meta-analyses) submitted to any journal in *The Lancet* family must include a panel putting their research into context with previous work in the format outlined below (see *Lancet* 2014; **384**: 2176–77, for the original rationale). This panel should not contain references. Editors will use this information at the first assessment stage and peer reviewers will be specifically asked to check the content and accuracy

WHO's International Clinical Trial Registry Platform
https://www.who.int/clinical-trials-registry-platform

Clinical trials
http://clinicaltrials.gov

ICMJE recommendations
http://icmje.org/recommendations/browse/publishing-and-editorial-issues/clinical-trial-registration.html

CONSORT 2025 guidelines
https://www.consort-spirit.org/

Formatting guidelines for randomised trials
https://www.thelancet.com/for-authors/forms?section=rct

CONSORT extended guidelines
http://www.consort-statement.org/extensions/extensions/

CONSORT-C Extension guidelines
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STARD guidelines
http://www.equator-network.org/reporting-guidelines/stard/

STROBE statement
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STREGA guidelines
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PRISMA guidelines
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Formatting guidelines for meta-analyses
https://www.thelancet.com/for-authors/forms?section=meta-analysis

GATHER statement
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CONSORT-AI Extension guidelines
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SPRIT-AI Extension guidelines
https://doi.org/10.1016/S2589-7500(20)30219-3

To find reporting guidelines, see
http://www.equator-network.org

Human Gene Organisation
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MIAME guidelines
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This section should include a description of all the evidence that the authors considered before undertaking this study. Authors should briefly state: the sources (databases, journal or book reference lists, etc) searched; the criteria used to include or exclude studies (including the exact start and end dates of the search), which should not be limited to English language publications; the search terms used; the quality (risk of bias) of that evidence; and the pooled estimate derived from meta-analysis of the evidence, if appropriate.

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- Where the data will be made available (including complete URLs or email addresses if relevant);
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- See **Conflicts of Interest** guidelines for comments

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- Letters for publication in the journal must reach us within 8 weeks of the online publication date of the original item and should be no longer than 400 words
- Letters of general interest, unlinked to items published in the journal, can be up to 400 words long
- Letters are not usually peer reviewed, but they will be subject to editorial scrutiny before any decision to publish is made; we might consult authors of the original publication for advice; and occasionally invite formal replies from the authors of the original publication for inclusion in the journal alongside a letter
- Only one table or figure is permitted, and there should be no more than five references and five authors
- All accepted letters are edited, and proofs will be sent out to authors before publication

Corrections

- Any substantial error in any article published in *The Lancet Public Health* should be corrected as soon as possible. Blame is not apportioned; the important thing is to set the record straight.
- The *Lancet* journals have a policy for types of errors that we do and do not correct. We will always correct any error affecting a non-proprietory drug name, dose, or unit, any numerical error in the results, or any factual error in the interpretation of results. Authorship format changes after publication to facilitate a different visualisation in MEDLINE/PubMed will not be done.

Green section (Reviews, Viewpoints, Health Policy, Commission)

Reviews

Most reviews are commissioned, but unsolicited short outlines (300–

400 words) can be directed to the Editor. If you have already written the paper, please submit it for consideration via our online system.

- Reviews should be a definitive overview of a major topic connected with public health
- Manuscripts will be assessed in-house and those judged suitable will be peer reviewed before an editorial decision is made
- Reviews should be no more than 4500 words, with a maximum of 75 references
- References selected for publication should be chosen for their importance, ease of access, and for the "further reading" opportunities they provide; citations to papers published in non-peer-reviewed supplements are discouraged. In addition to references, authors should consider supplying a short list of useful websites where readers can find further information on the subject
- A 150 word unstructured summary should be included. Use of up to 5–6 illustrations is encouraged to aid the reader
- Complete transparency about the choice of material included is important to any Review paper. Therefore, all Reviews should include a brief section entitled "Search strategy and selection criteria" stating the sources (including databases, MeSH and free text search terms and filters, and reference lists from journals or books) of the material covered, and the criteria used to include or exclude studies. Citations to papers published in non-peer-reviewed supplements are discouraged. Since these papers should be comprehensive, we encourage citation of publications in non-English languages.

Systematic reviews should be prepared according to the PRISMA guidelines

Viewpoint

Viewpoints are opinion pieces that use the best evidence to develop a robust argument on a topic of immediate relevance to public health. They should have a novel and clear point to make, with the aim of provoking transformational thinking at a high level. Length guidelines are up to 2500 words and with a maximum of 30 references.

Health Policy

Manuscripts considered for this section are narrative reviews (not original research) and should follow the same guidelines as a Review. These papers should cover developments in public health related to policy, health systems, or economics. Other related topics will be considered. Please contact the Editor before submitting to ensure the proposed topic is suitable.

Commissions

Topics for *The Lancet Public Health* Commissions are selected by our editors, who work with academic partners to identify the most pressing issues in public health with the aim of producing recommendations to change policy or practice. All Commissions are academic publications and are subject to the same rigorous peer review process as all other research papers published in our journals. *The Lancet Public Health* does not provide direct financial support to Commissioners for the research or writing of the reports. Funding is sought directly by authors, with oversight from our editors.

Series

Series are collections of papers (usually between three and five) on a broad topic within the public health field. They are generally commissioned by the editors, but suggestions are welcome by email. We would not consider a large collection of papers on a narrow topic—eg, a single public health programme. This type of collection is better suited to a journal supplement (*The Lancet Public Health* does not publish supplements).

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Formatting guidelines

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- Numbers one to ten are written out in words unless they are used as a unit of measurement, except in figures and tables
- Use single hard-returns to separate paragraphs. Do not use tabs or indents to start a paragraph
- Do not use the automated features of your software, such as hyphenation, endnotes, headers, or footers (especially for references). Please use page numbering
- Guidelines on formatting tables are available in the artwork guidelines

Formatting guidelines for text, tables, and figures
Guidelines on formatting of text, tables, and figures can be found at **https://www.thelancet.com/pb/assets/raw/Lancet//authors/artwork-guidelines.pdf**

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- Cite references in the text sequentially in the Vancouver numbering style, as a superscripted number after any punctuation mark. For example:

"...as reported by Saito and colleagues.[15]"

- Two references are cited separated by a comma, with no space. Three or more consecutive references are given as a range with an en rule. To create an en rule on a PC: hold down CTRL key and minus sign on the number pad, or on a Mac: ALT hyphen
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"15[tab]Saito N, Ebara S, Ohotsuka K, Kumeta J, Takaoka K. Natural history of scoliosis in spastic cerebral palsy. *Lancet* 1998; **351:** 1687–[en rule]92."

- Give any subpart to the title of the article
- If there are six authors or fewer, give all six in the form: surname space initials comma
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Drug names

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- We encourage use of neuroscience-based nomenclature for psychotropic drugs

References

- Vancouver style—eg,

  Smith A, Jones B, Clements S. Clinical transplantation of tissue-engineered airway. *Lancet* 2008; **372:** 1201–09.

  Hourigan P. Ankle injuries. In: Chan D, ed. Sports medicine. London: *Elsevier,* 2008: 230–47.
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Drug names
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